



# Alcohol, Illicit Substances, Non-Prescribed Substances Management on the Inpatient Wards Procedure

## 1. Scope

| Site                    | Operational Area       | Applicable to      |
|-------------------------|------------------------|--------------------|
| Armadale Health Service | Mental Health Services | All clinical staff |

## 2. Purpose

The consumption, use, possession and / or distribution of alcohol and / or illicit substance and / or medications not prescribed by the treating team by a mental health inpatient or visitor of the Armadale Mental Health Service (AMHS) is not permitted.

If there is reasonable concern that an inpatient has consumed alcohol and or illicit substance, the consumer should be requested to submit to an appropriate test i.e. Breathalyser or UDS as appropriate.

Consumers and visitors will be informed at the point of admission or at the earliest opportunity:

- That the consumption, use, possession or distribution of alcohol and / or illicit substances and / or medications not prescribed by the treating team is prohibited and will not be tolerated;
- Testing may be undertaken in situations where there is a strong suspicion of alcohol / illicit substance use and where such could impact on the consumer's mental state or could compromise the consumer's treatment plan

## 3. Procedure

### 3.1 Consumption or use of alcohol or illicit substance or medications not prescribed by the treating team:

- If a consumer is suspected of or known to have consumed alcohol or use prohibited or illicit substances or medications not prescribed by the treating team, this is to be reported to the Shift Coordinator.
- The consumer is to be observed for identifying signs or symptoms of consumption or use.
- The safety of the consumer, other consumers, visitors and staff must be ensured.
- The Shift Coordinator is to inform the Duty Medical Officer or treating team doctor, Nurse Unit Manager (NUM)/Associate Nurse Unit Manager (ANUM)/ Afterhours Mental Health Clinical Nurse Specialist (AH MH CNS) immediately upon receipt of the report from the staff member.
- The Duty Medical Officer / treating team doctor is to review the consumer. Based on medical and risk assessments, the reviewing doctor may decide to amend the Management Plan as required.

- The consumer's level of toxicity is to be established via blood sample, urinary drug screen and/or breathalyser.
- If the consumer is assessed to be potentially medically compromised the Duty Medical Officer / treating team doctor is to liaise with the Armadale Health Service On-Call Medical Officer to request assessment and further treatment as required.
- The Shift Coordinator / Duty Medical Officer / treating team doctor will determine if close and continuous supportive observations are to be initiated. Refer to the AMHS policy [Observations: Safe and Supportive](#).
- A Code Blue should be activated if the consumer's physical state is acutely compromised. Refer to the [Code Blue – Medical Emergency Action Card](#). The Shift Coordinator is to alert all staff on the ward of the incident.
- The Shift Coordinator is to inform the NUM / ANUM/ AH MH CNS, and Security, if necessary, of the incident.
- The incident, observations and assessments are to be documented in the consumer's health record in the chronological order of events.
- If appropriate, a brief intervention discussion should occur with the consumer in relation to harm of substances in their mental health recovery.
- The incident is to be reported via Datix CIMS.

### 3.2 Possession and/or distribution of alcohol or illicit substance:

- Suspected or known possession and/or distribution of alcohol or illicit substance is to be reported to the Shift Coordinator.
- The consumer is to be observed for confirmation of distribution and/or possession of alcohol, illicit substances.
- The safety of the consumer, other consumers, visitors and staff must be ensured.
- The Shift Coordinator is to alert all staff on the ward and ensure all staff understand that they are not to approach the consumer, alone.
- The Shift Coordinator is to inform the NUM/ANUM/ AH MH CNS and Security of the incident.

#### **Visitors in possession of alcohol or illicit substances.**

Please refer to the conditions of visitation in the [AKG Visitor Access Procedure](#).

If possession of alcohol, illicit substance or medications not prescribed by the treating team is confirmed, the Shift Coordinator with another staff member present and/or Security staff present is to request that the consumer submits the identified substance:

**Alcohol:** the consumer is to be asked to remove the alcoholic beverage from the hospital, including the grounds, either by themselves, or through a visitor, carer or other personal support person.

If the consumer does not comply with the request to have the alcohol beverage removed, Security will be requested to remove the alcohol beverage from the clinical area and store it appropriately.

Refer to EMHS policy: [Alcohol and Prohibited Substances](#).

**Illicit substance:** The Shift Coordinator with another staff member present and/or Security staff present, is to request that the consumer submits the illicit substance.

If the consumer submits the illicit substance, it is to be placed in an envelope and sealed.

The following information is to be recorded on the envelope:

- Date and time (24 hour format) the illicit substance / non-prescribed medications was placed in the envelope;
- Printed names and designations of staff members who placed the illicit substance in the envelope; and staff who were in attendance at the time;
- Signatures of each of the staff members whose names are recorded on the envelope;
- Patient label.
- Envelope is to be handled by security for storage and/or WA Police intervention.

If the consumer refuses to submit the illicit substance, staff are to observe the consumer until Security staff arrive.

Staff will provide Security staff with the demographic details of the consumer. Security staff will contact WA Police.

A Security Incident Form and incident notification via Datix CIMS are to be completed.

The incident, observations and actions taken are to be documented in the consumer's health record in the chronological order of events.

### **3.3 Possession and distribution of medications not prescribed by the treating team:**

- Suspected or known possession and/or distribution of medications not prescribed by the treating team is to be reported to the Shift Coordinator.
- The consumer is to be observed for confirmation of distribution and/or possession of medications not prescribed by the treating team.
- The safety of the consumer, other consumers, visitors and staff must be ensured.
- The Shift Coordinator is to alert all staff on the ward and ensure all staff understands that they are not to approach the consumer, alone.
- Once confirmed that the consumer is in possession of medications not prescribed by the treating team, the consumer is to be informed that the medications will be removed, and the treating team will determine medication regimens whilst the consumer is an inpatient.
- The non-prescribed medication, will be logged as property as per the [Patient Property in Mental Health Wards MEN-PRO-0344-16](#).

### **3.4 Testing Consumers for alcohol or illicit substance use**

- Patients on admission to the ward will be requested to provide a routine Urine Drug Screen (UDS). Refer to the [Assessment Requirements for Mental Health Consumers MEN-PRO0046/15](#)
- Testing may be undertaken in situations where there is strong suspicion of alcohol / illicit substance use by a consumer and where such use could impact on the consumer's mental state, physical state or compromise the consumer's treatment plan.
- Relevant medical staff must be involved in the decision making process regarding testing the consumer.
- The consumer must be provided with an explanation of the reason for the testing,

including potential outcomes; and this must be documented in the consumer's health record.

- The consumer's consent must be obtained prior to testing for alcohol or illicit substance use.
- If the consumer refuses to be tested, the consumer's behaviour, demeanour and actions will be documented in the health record and discussed with the treating team at the earliest point.
- The Shift Coordinator / Duty Medical Officer / treating team doctor will determine if close and continuous supportive observations are to be initiated. Refer to the [Observations: Safe and Supportive Procedure MEN-PRO-0310-16](#).

### 3.5 Searching a consumer and/or their property

Where staff hold a reasonable concern that a consumer may be in possession of items considered a danger to themselves or others, a search of that consumer may be undertaken as per the [Patient Property in Mental Health Wards Procedure MEN-PRO-0344-16](#).

## 4. Decision to Discharge

The Shift Coordinator may consult with the treating team doctor / Duty Medical Officer regarding the decision to discharge the consumer/s in the possession of; and/or responsible for the distribution of alcohol / illicit substances / medications not prescribed by the treating team.

The treating team doctor / Duty Medical Officer will consider as a minimum, the consumer's current:

- Mental state;
- Risk assessment;
- Treatment Support and Discharge Plan (TSDP)

If the decision to discharge the consumer is made, the requirements of the [Discharge from the Mental Health Inpatient Wards and Community Mental Health Teams Procedure MEN-PRO-0128-15](#) are to be met.

## 5. Legislative context

The following documents are required to give effect to this procedure:

- *Mental Health Act 2014*
- Health Department of Western Australia (HDWA) [Clinical Incident Management Policy MP 0122/19](#)
- HDWA [Recognising and Responding to Acute Deterioration Policy MP 0086/18](#)
- HDWA [Consent to Treatment Policy OD 0657/16](#)
- East Metropolitan Health Service (EMHS) [Alcohol and Prohibited Substances EMHS: 112](#)
- EMHS [Searching of Mental Health Patients Belongings and Property EMHS: 153](#)
- EMHS [Testing Mental Health Inpatients for Alcohol and / or Illicit Substances Use and / or Medications not Prescribed by the Treating Team EMHS: 151](#)

## 6. Supporting information

The following documents support the implementation of this procedure:

- [Discharge from the Mental Health Inpatient Wards and Community Mental Health Teams Procedure MEN-PRO-0128-15](#)
- [Patient Property in Mental Health Wards Procedure MEN-PRO-0344-16](#)
- [Observations: Safe and Supportive Procedure MEN-PRO-0310-16.](#)
- [Alcohol, Tobacco and Other Drug – Patient Management MEN-PRO-0023-10](#)
- [Clinical Incident Management AKG-PRO-0087-17](#)
- [Recognition and Response to Acute Deterioration AKG-POL-0388-14](#)

## 7. Compliance, monitoring and evaluation

Compliance against this procedure will be monitored by the Mental Health Safety and Quality Departmental Meeting.

Monitoring activities will include:

- Reported clinical incidents via the Datix Clinical Incident Management System (CIMS)

## 8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Action 1.30 Safe environment for the delivery of care
- The health service organisation:
  - a. Identifies service areas that have a high risk of unpredictable behaviours and develops strategies to minimise the risks of harm for patients, carers, families, consumers and the workforce
  - b. Provides access to a calm and quiet environment when it is clinically required
- Standard 8 – Action 8.5 Recognising acute deterioration
- The health service organisation has processes for clinicians to recognise acute deterioration in mental state that require clinicians to:
  - a. Monitor patients at risk of acute deterioration in mental state, including patients at risk of developing delirium
  - b. Include the person's known early warning signs of deterioration in mental state in their individualised monitoring plan
  - c. Assess possible causes of acute deterioration in mental state, including delirium, when changes in behaviour, cognitive function, perception, physical function or emotional state are observed or reported
  - d. Determine the required level of observation
  - e. Document and communicate observed or reported changes in mental state

National Standards for Mental Health Services

- Criteria 2.6
- The MHS meets their legal occupational health and safety obligations to provide a safe workplace and environment.

- Criteria 2.11
- The MHS conducts risk assessment of consumers throughout all stages of the care continuum, including consumers who are being formally discharged from the service, exiting the service temporarily and/or are transferred to another service.
- Criteria 2.12
- The MHS conducts regular reviews of safety in all MHS settings, including an environmental appraisal for safety to minimise risk for consumers, carers, families, visitors and staff.

## 9. Policy owner (relating to these procedures)

### Service Director MH

Enquiries relating to this procedure may be directed to:

Title: Nurse Unit Manager  
Department: Mental Health Services  
Email: Rijo.Joseph@health.wa.gov.au

## 10. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

| Version | Effective from | Effective to | Amendment(s)                       |
|---------|----------------|--------------|------------------------------------|
| V6      | 14/11/2023     | 14/11/2026   | Scheduled review - Minor amendment |

## 11. Authorisation

This procedure has been authorised and issued by the Service Director Mental Health.

|                    |                                                |
|--------------------|------------------------------------------------|
| Authorised by      | Karen Elliott – Service Director Mental Health |
| Authorisation date | 14/11/2023                                     |
| Published date     | 14/11/2023                                     |